

February 22, 2026

HHS Office of the Deputy Secretary

RE: HHS Health Sector AI RFI

Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care [RIN 0955-AA13]

This comment is submitted anonymously by a group of healthcare technology professionals and clinical innovators. We have chosen to submit these insights without individual or organizational identification to ensure the feedback remains focused on the structural barriers to innovation. Our intent is to provide a candid assessment of the current regulatory and technical landscape without drawing undue attention to specific partnerships or individual organizations from larger entities that may be impacted by the policy changes suggested herein.

Our perspective is informed by extensive experience in deploying AI-driven patient navigation across diverse health systems. We appreciate the opportunity to provide feedback on how regulatory frameworks and interoperability standards can better support the integration of agentic AI to improve patient access and health system efficiency.

I. Regulation

The current regulatory environment could evolve to better address the "hidden" barriers to AI adoption. While clinical safety remains paramount, a significant bottleneck exists in the accessibility of the logic and administrative data required to navigate patients effectively through complex health systems.

- **Transparency and API Mandates:** HHS might consider interpreting "information blocking" regulations (e.g., 45 CFR Part 171) to include not just patient clinical records, but also the administrative decision trees and scheduling logic developed by health systems. Currently, these logic "knowledge bases" are often locked within proprietary EHR systems, which can prevent AI agents from accurately routing patients to the optimal care setting.
- **Proportionate Oversight:** We suggest a regulatory posture that prioritizes explainability. AI navigation tools could be encouraged to demonstrate that their outputs are "tunable" by the health system's clinical leadership, helping ensure the AI adheres to the local standard of care.

II. Reimbursement

To further accelerate AI adoption, payment policy could move beyond "fee-for-service" logic, which may lack the agility to cover digital navigation.

- **Incentivizing High-Value Navigation:** HHS could prioritize payment models that reward the reduction of "leakage" and "misrouting." When a patient is navigated to the correct care level (such as Urgent Care instead of the Emergency Room) by an AI agent,

the payer realizes immediate savings. Reimbursement models could be designed to reflect this value.

- **Liability and Coverage:** CMS could provide clearer pathways for the "professional fee" component of AI-augmented triage. This would help ensure that health systems are not financially penalized when they transition from manual, call-center-based workflows to more efficient, clinically-validated AI agents.

III. Research & Development

HHS might focus R&D investment on Implementation Science, specifically regarding how AI integrates with existing healthcare "gatekeeping" functions.

- **Public-Private Partnerships:** We recommend that HHS leverage CRADAs to test agentic navigation within large-scale environments like the Defense Health Agency (DHA). These environments serve as ideal "sandboxes" for proving how AI can standardize referral qualifications across a massive and geographically diverse population.

Responses to Specific Questions

1. Biggest Barriers to Private Sector Innovation The single largest barrier to innovation in patient navigation appears to be the lack of interoperability with administrative "knowledge" data. While clinical data like labs and notes is increasingly accessible via FHIR APIs, the scheduling logic and specialty-care decision trees often remain siloed. EHR vendors sometimes decline to expose APIs to these rules, even when requested by the health systems who "own" that knowledge. This can force AI developers to build redundant, manual workarounds, which potentially slows deployment and increases costs.

2. Regulatory and Programmatic Design Changes HHS could consider prioritizing the following updates:

- **Update 45 CFR § 170.315:** Expand certification criteria to suggest that EHR vendors expose APIs for administrative scheduling rules and provider-logic trees.
- **Enforce Information Blocking Rules:** Clarify that refusing to provide API access to a health system's own clinical or administrative logic (when requested by that system for a third-party AI integration) may constitute a barrier to innovation.

6. Successes and Shortfalls of Deployed AI

- **Successes:** Agentic navigation has successfully helped patients across all 50 states navigate to the correct level of care, reducing the burden on overstretched call centers at several of the nation's largest Integrated Delivery Networks (IDNs) and regional health systems..
- **Shortfalls:** These tools can fall short of their full potential when they cannot "talk" to the internal scheduling logic of the EHR in real-time. This often necessitates a "safe handoff"

(asynchronous) rather than a "closed-loop" (real-time) booking, which can degrade the patient experience.

7. Influence and Administrative Hurdles Within health systems, Chief Information Officers (CIOs) and Chief Medical Officers (CMOs) typically hold the most influence. However, the primary administrative hurdle is the reconciliation of scheduling rules. Health systems go through rigorous processes to define how a patient should be routed for sub-specialty care. When an EHR vendor does not expose the API for these rules, the AI implementation requires a "re-build" of that logic, creating significant administrative duplication.

8. Enhanced Interoperability Interoperability could move beyond data types (what the patient has) to logic types (where the patient should go). Specifically:

- **Data Standards:** HHS might support the development of standards for "Care Routing Logic" to complement existing FHIR clinical resources.
- **Benchmarking:** Standardized benchmarking for "Triage Accuracy" would allow the private sector to demonstrate that AI-driven navigation is as safe as (or safer than) traditional nurse advice lines.

9. Patient and Caregiver Concerns Patients primarily seek clarity and convenience. They can be overwhelmed by "choice paralysis" when seeking care, and AI agents address this by providing a single "digital front door." However, caregivers remain concerned about "dead ends" where an AI provides a recommendation but cannot facilitate the actual appointment due to the API barriers mentioned above.

Respectfully submitted,

Anonymous Industry Stakeholders